INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Trastuzumab subcutaneous (SC) · early breast cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00285
Version 4

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, to complete one year total. Previous intravenous (IV) or combination-treatment trastuzumab counts towards that year; a route switch does not start a new year.

Treatment stages

01 **Current HER2 treatment**
Every 3 weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Trastuzumab subcutaneous (SC) · early breast cancer · National Cancer Control Programme (NCCP) 00285

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report breathlessness, swelling, rapid weight gain, palpitations or persistent diarrhoea; tell staff immediately about a reaction during treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

The medicine in this card

Trastuzumab under the skin is part of treatment to reduce recurrence in HER2-positive early breast cancer. The wider plan can include chemotherapy, surgery, radiotherapy and endocrine treatment when indicated.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Keep echo appointments. Report breathing changes, swelling, allergic symptoms or a painful / spreading injection-site reaction.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

Setting and duration

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, to complete one year total. Previous intravenous (IV) or combination-treatment trastuzumab counts towards that year; a route switch does not start a new year.

Medicine identity

This card describes trastuzumab. Pertuzumab, taxanes, endocrine medicines and antibody–drug conjugates require their own treatment instructions.

Trastuzumab formulation evidence

This explains the study setting and its meaning. Ask the team how the evidence applies to you.

Longer follow-up found similar outcomes with the two formulations. This study helps explain the choice of route; it does not predict your own outcome.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Trastuzumab SC · early breast cancer · NCCP 00285

Where this treatment fits

Trastuzumab SC · early breast cancer

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 3 WEEKS

Current HER2 treatment

Repeated cycles

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, to complete one year total. Previous intravenous (IV) or combination-treatment trastuzumab counts towards that year; a route switch does not start a new year.

Early and metastatic pathways have different aims and endpoints. The team confirms the applicable plan.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. NCCP remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment. After stopping: six-monthly to 24 months; after anthracyclines in early disease, yearly to 5 years or longer if needed.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Blood tests	Baseline full blood count (FBC), renal and liver profile	Additional tests if needed	Repeat before this cycle	Repeat before this cycle	Every 6 weeks
electrocardiogram (ECG) / rhythm	Baseline ECG in EMA cardiac assessment; it does not replace echo.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Record surgery, breast imaging and follow-up plan	Assess symptoms at visits	Additional imaging only if clinically indicated	Routine response scans are not implied after curative treatment	Continue breast surveillance; routine metastatic scans or tumour markers are not indicated in symptom-free early disease.
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	If endocrine / ovarian suppression is planned: assess need	Not a routine trastuzumab test alone	Timing depends on risk / result	Separate from the echo schedule	DEXA and repeat timing depend on endocrine treatment, fracture risk and results.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00285 v4 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · EMA Herceptin product information · retrieved 25 October 2026

THE WHOLE TREATMENT JOURNEY

Trastuzumab SC · early breast cancer: treatment and checks

HER2-positive early breast cancer · SC component

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00285	Confirm setting, prior doses and formulation	SC trastuzumab every 3 weeks	Review benefit, symptoms and heart function	Complete one year total of trastuzumab	Complete the planned total year, including prior IV doses, or change earlier for recurrence/ toxicity.
Surgery EARLY	Early: surgery may precede or follow initial systemic therapy	Neoadjuvant route: reassess before planned surgery	multidisciplinary team (MDT) reviews recovery and pathology	Early: continue prescribed postoperative HER2 plan	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Early: usually after chemotherapy; coordinate surgery	May overlap trastuzumab continuation if agreed	Early: radiotherapy (RT) may overlap trastuzumab continuation, when agreed.	Radiation effects and surveillance follow the local plan
Blood tests NCCP 00285	Baseline FBC, renal, liver	Every 6 weeks	Week 12 is another six-week blood-check point for trastuzumab alone; companion therapy may require more frequent tests.	Every 6 weeks	Follow abnormalities and the next treatment plan
Heart checks NCCP 00285 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Trastuzumab: six-monthly cardiac checks to 24 months after stopping; see follow-up detail.
Scans / markers EARLY · METASTATIC · MARKERS	Record the breast imaging and postoperative follow-up plan	Review new symptoms at every visit	No routine metastatic response scan for symptom-free early disease	Continue breast surveillance and symptom-led assessment	Additional systemic scans and tumour markers are not routine in symptom-free early disease.
Endocrine / bone health EARLY · BONE	Check hormone receptor status and endocrine indication	Endocrine timing coordinated with the wider plan	DEXA if indicated by endocrine / fracture risk	May overlap trastuzumab when clinically appropriate	Longer endocrine and bone follow-up can outlast HER2 treatment

What each check is for

1

Blood tests

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time)

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

2

Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

3

Cancer imaging and markers

Early disease: breast surveillance arranged after local treatment; routine computed tomography (CT) / tumour markers are not recommended for asymptomatic recurrence screening.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

4

Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

SC trastuzumab needs no IV line for this medicine. Another medicine may still need a cannula or port.

STEP 3

Treatment and observation

Injection over 2–5 minutes into alternate thighs. Observation is 30 minutes after the first injection and 15 minutes after subsequent injections, subject to approved local policy.

STEP 4

After the visit

Contact the unit to rearrange a missed dose. SC doses remain at least 3 weeks apart; no loading dose. This medicine does not routinely require a take-home pump.

Injection site

Alternate thighs and avoid tender, bruised, red or hardened skin. Tell the nurse about a persistent or worsening local reaction.

Before treatment and between visits

The team should arrange and review the tests.

Blood checks

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time)

Cardiac checks

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Baseline cardiac assessment includes ECG in EMA trastuzumab information. Further ECGs follow symptoms / cardiac plan; an ECG does not replace an echo.

After stopping

EMA trastuzumab information: cardiac assessment every 6 months after stopping, until 24 months; after anthracyclines in early disease, yearly to 5 years or longer if function falls.

The wider pathway

Coordinate cancer medicines with local treatment and follow-up.

Scans and markers

Early disease: breast surveillance arranged after local treatment; routine CT / tumour markers are not recommended for asymptomatic recurrence screening.

Radiotherapy relationship

If postoperative radiotherapy is indicated, it usually follows chemotherapy and may overlap with trastuzumab continuation under the MDT plan. The radiation team assesses heart / lung exposure. Endocrine therapy may also overlap when indicated.

Bone health

If an aromatase inhibitor or ovarian suppression is planned, assess fracture risk and DEXA need. Repeat timing depends on results and bone treatment; this is not an automatic HER2-therapy scan.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Trastuzumab SC · early breast cancer · NCCP 00285

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
SC injection	 **Day 1** Planned dose
Administration reaction	 **Day 1** During / after visit; report promptly
Fatigue / bowel changes	 **Days 1–21** Timing and severity vary
Heart / breathing symptoms	 **Days 1–21** Report on any day

No date entry needed: This is an orientation map; it does not calculate a personal appointment calendar.

Companion medicines: If another cancer medicine is prescribed, its side effects, tests and supportive medicines need its own plan.

Delayed treatment: Blood tests and echoes follow clinical due windows. A postponed dose does not automatically postpone monitoring.

Why these bands stay broad: Treatment days show the planned schedule. Symptom bands are broad possibilities, not measured risk curves or guaranteed safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00285 v4 · official regimen ↗

There are no guaranteed "safe days". New breathlessness, chest pain, collapse, severe allergic symptoms or sudden serious illness need urgent assessment on any day.

Between-treatment notes

Day 1–21: Your reference cycle

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, to complete one year total. Previous IV or combination-treatment trastuzumab counts towards that year; a route switch does not start a new year. The team confirms each dose after the relevant checks. Contact them about a missed visit or new symptoms.

Support around the medicines

1 Support for this medicine

Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.

2 Blood and heart appointments

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time). Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles.

3 Local treatment and endocrine therapy

Surgery and radiotherapy are coordinated by the MDT. When endocrine therapy is indicated, it has its own schedule and bone-health plan.

4 Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Questions worth asking

When is my next echo?

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Existing HER2 exposure and delays affect cycle numbering; the team should give the actual due window.

Does a tumour marker replace a scan?

No. In metastatic disease it may add information to symptoms and imaging. It should not alone determine treatment changes. In symptom-free early disease it is not routine recurrence screening.

Try explaining the plan back: What is the purpose of your treatment? Which heart or blood test is due next, and which team arranges and reviews it?

Trastuzumab

- Heart pumping-function reduction / heart failure
- Allergic or administration-related reactions
- Injection-site pain, redness or swelling
- Diarrhoea, fatigue and rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Trastuzumab SC · early breast cancer

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.
- Injection over 2–5 minutes into alternate thighs. Observation is 30 minutes after the first injection and 15 minutes after subsequent injections, subject to approved local policy.
- Contact the unit to rearrange a missed dose. SC doses remain at least 3 weeks apart; no loading dose.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00285 · VERSION 4

Trastuzumab SC · early breast cancer

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Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

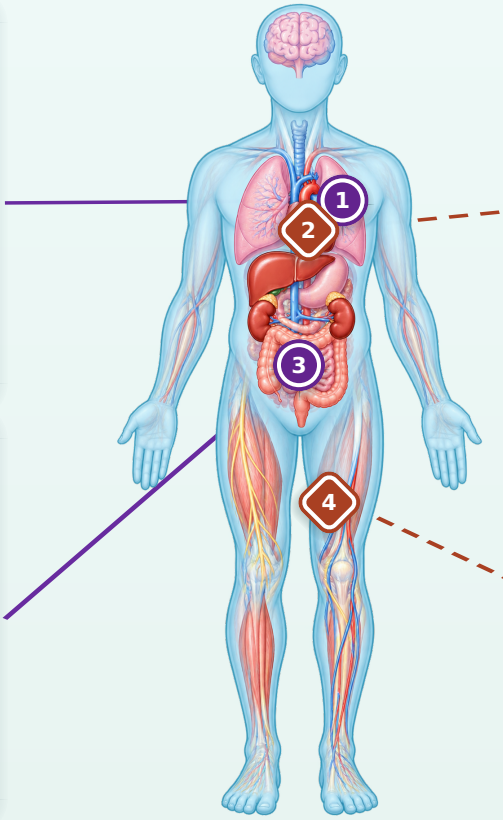
Trastuzumab

New cough or breathlessness: seek prompt assessment.

2 Heart

Trastuzumab

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.



3 Bowel / fatigue

Trastuzumab

Report persistent diarrhoea, dehydration or fatigue affecting daily activity.

4 Injection site

SC trastuzumab

Report painful, spreading redness or swelling where the injection was given.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

SACTCHECK · CHAPTER 04

Sources and further information

Trastuzumab subcutaneous (SC) · early breast cancer · National Cancer Control Programme (NCCP) 00285

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00285 v4 · official regimen ↗**
   https://healthservice.hse.ie/documents/6700/285_v4_Trastuzumab_SC_21days_EBC.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
6. **EMA Herceptin product information · retrieved 25 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
7. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NCCP NCIS antiemetics v8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **eviQ · implanted port patient information**
    https://www.eviq.org.au/p/507
12. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
13. **HannaH · formulation comparison (2012)**
    https://pubmed.ncbi.nlm.nih.gov/22884505/
14. **HannaH final six-year analysis (2019)**
    https://jamanetwork.com/journals/jamaoncology/fullarticle/2730632